

REG-02: State Laws on AI in Utilization Review (states relevant to Bellcourt)

Training summary prepared for the Bellcourt case study. Simplified for learning purposes; not legal advice and not a substitute for the source text.

A wave of state legislation in 2025 and 2026 restricts the use of AI and automated tools in coverage decisions. The common pattern: AI may assist, but a qualified human clinician must make any adverse medical-necessity decision. State insurance laws apply to insured products (Riverbend) and generally NOT to self-funded ERISA plans, although sponsors may adopt the same standards voluntarily.

State	Summary of requirement	Effective	Relevance to Bellcourt
Arizona	A licensed medical director must personally review and sign any denial involving medical necessity; AI cannot be the sole basis.	July 1, 2026	Riverbend MA members in AZ
Texas	Written disclosure to patients when AI is used in connection with their health care services; utilization review determinations made by physicians.	January 1, 2026	Riverbend MA members in TX
Georgia	Coverage decisions may not be based solely on AI or other software tools; human clinical review required before denial.	January 1, 2027	Riverbend Marketplace (GA)
Colorado	AI used in utilization review must consider individual clinical history, be audited periodically for accuracy, and not be applied in a discriminatory way; qualified professional must review medical-necessity denials.	January 1, 2027	No insured Bellcourt business in CO (Brightwater is self-funded)
Illinois	Algorithmic downcoding of claims restricted; human review of downcoding required. Self-insured ERISA plans excluded.	January 1, 2028	Claims (not prior authorization)
California	Physicians Make Decisions Act: medical-necessity denials must be made by a licensed physician.	January 1, 2025	No current Bellcourt business

Note: at least ten states had enacted or advanced similar laws by mid-2026, and each defines 'human review' differently.